

Constipation



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Drug Information

GENERIC NAME	USUAL STARTING DOSE AND RANGE	STRENGTHS AND FORMULATIONS	COMMENTS	CRUSH/ OPEN?
STIMULANT LAXATIVES				
Senna (Ex-Lax)	Initial: 1-8 tablets PO divided QD-BID PRN MDD: 8 tablets or 40ml/ day	Tablet: 8.6 mg sennosides Oral solution: 8.8mg/5ml	- Onset of action 6-12 hours - Drug of choice for opioid induced constipation - The typical maximum daily dose is occasionally exceeded in the hospice setting	Y
Senna/Docusate Sodium (Senna-S)	Initial: 1-2 tablets PO daily to BID MDD: 8 tablets/day	Tablet: Docusate 50mg/ Senna 8.6mg	- See individual drugs for onset of action - Stimulant laxative + stool softener combo - May crush, although tablets have a poor taste when crushed	Y
Bisacodyl (Dulcolax)	Initial (tablets): 1-8 tablets PO QD PRN Initial (suppository): 1 suppository PR QD PRN MDD (tablets): 40mg MDD (suppository): 20mg	Tablet: 5 mg Suppository: 10 mg	- Suppository onset of action 15 minutes to 2 hours. - Tablet onset of action 6 to 10 hours - Stimulant laxative - Can be used daily or PRN	Y
OSMOTIC LAXATIVES				
Magnesium Citrate (Citroma)	Initial: ½ to 1 bottle (150-300mL) PO PRN MDD: 1 bottle/day	Oral solution: 1.745g/30ml	- Onset of action 30 minutes to 6 hours - Contraindicated w/ renal impairment - Avoid in patients with heart failure due to high salt content and potential to worsen fluid overload - May be given as single or divided dose - Reported to taste better when cold - Keep chilled in refrigerator after opening	-
Magnesium Hydroxide (Milk of Magnesia)	Initial (suspension): 30-60ml QD Initial (tablet): 1-8 tablets once daily or in divided doses MDD (suspension): 60ml/ day MDD (tablet): 8 tablets/day	Chewable tablet: 311mg Oral suspension 400mg/5mL, 800mg/5mL	- Onset of action in 30 minutes to 3 hours - Contraindicated w/ renal impairment - Consider milk of magnesia if senna in combination with docusate is ineffective	Y

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OSMOTIC LAXATIVES (CONTINUED)				
Magnesium Oxide (Uro-mag, Maox)	Initial: 1-8 tablets PO QHS or in divided doses MDD: 8 tablets/day	Tablet: 400mg	Contraindicated w/ renal impairment	Y
Magnesium Sulfate (Epsom salt)	Initial: 2 teaspoonsful of granules dissolved in water once daily MDD: 6 teaspoonsful/day	Granules	- Dissolve in warm water and take with extra fluid - Contraindicated w/ renal impairment	-
Sodium Phosphates (Fleet Enema)	Initial: 1 enema PR x 1 dose MDD: 2 enemas /day	Enema: 5.6 gm/dose	- Onset of action 2 to 15 min. - Use with caution in patients w/ renal impairment - Avoid in patients with heart failure due to high salt content and potential to worsen fluid overload	-
Glycerin Suppositories (Fleet glycerin, Pedia-Lax)	Initial: 1 suppository PR daily to BID PRN MDD: 2 suppositories	Suppository: 1.2gm, 2gm, 2.1gm	- Onset of action 15 min. to 1 hr - To be used as needed for hard, dry stool - May cause "fatty" appearing stool	-
Lactulose (Enulose)	Initial: 15-30ml (10-20g) PO QD MDD: 240ml/day	Syrup: 10g/15ml	- Onset of action up to 24-48 hours - Laxative of choice if liver failure is present - Titrate to produce 3 to 4 soft stools per day, avoiding diarrhea for hepatic encephalopathy - Does not increase blood glucose - Associated with more cramping / bloating than other osmotic laxatives	-
Sorbitol	Initial: 15-60 mL PO divided QD-QID MDD: 240ml/day	Oral solution: 70%	- Excessively sweet taste - Does not increase blood glucose	-
Polyethylene glycol (Miralax)	Initial: 17g (one capful) in 8oz of fluid daily MDD: 34g/day	Powder	- Onset of action 24-96 hours - Patient must be adequately hydrated - May not be suitable for patients with volume intolerance - Preferred osmotic laxative in patients with renal impairment - Best tolerated osmotic laxative	-